

AI-enabled Population Health & Clinical Performance Command Center

Meridian Health System (synthetic) — Synthetic, Meridian/PHS-inspired pilot context. Not real confidential PHS data.

Recommendation

Fund the command center as one governed Strategic Move spanning provider and plan, sponsored by the Chief Population Health Officer, with CFO approval at the P4 funding gate.

Value at stake

Projected \$38.0M-\$61.0M per year across five levers.

- Avoidable admissions reduction: \$12.0M-\$19.0M (Chief Population Health Officer)
- Care-gap closure (HEDIS/STAR): \$8.0M-\$13.0M (VP Population Health Analytics)
- Quality-bonus realization: \$9.0M-\$14.0M (Health Plan COO)
- Risk-adjustment accuracy (RAF): \$5.0M-\$9.0M (Chief Actuary (Plan))
- Post-acute / readmission: \$4.0M-\$6.0M (Chief Medical Officer)

Phases & gates

P0 Originate - Originate brief: Frame the command-center bet across provider and plan; name sponsor, value range, and the first gate.

P1 Charter - Charter: Lock scope, decision rights, value hypothesis, and the evidence the Move must clear at each gate.

P2 Discover & Diagnose - Discovery brief: Diagnose the loaded baseline: gaps, denials, acuity, data readiness, and where AI realistically moves the metric.

P3 Design Future State - Solution & target-state architecture: Design the bronze/silver/gold lakehouse, the model portfolio, and the clinical-safety governance path.

P4 Roadmap & Business Case - Costed business case & value model: Sequence the waves, cost the platform and AMS, and produce the funding-gate business case.

P5 Mobilize & Handoff - Mobilization plan & value-measurement contract: Stand up the RACI, the operating cadence, and the value-measurement contract Control Tower will own.

Assumptions & guardrails

Value ranges are synthetic planning estimates pending CFO validation. Clinical models require AI

Governance Council clinical-safety sign-off. No figure is confidential PHS proof.